

PATIENT ADVISORY COUNCIL IMPLEMENTATION IN AN UNDERSERVED FAMILY MEDICINE RESIDENCY CONTINUITY CLINIC

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ABSTRACT

We will establish a bilingual English-Spanish Patient Advisory Council (PAC) to provide patients a structured voice to give feedback on their experience, shaping workflows, educational materials, and physician-patient relationships.

Our four key innovations:

1. Resident and Patient-led Facilitation
2. Bilingual, Equity-Centered Structure
3. Patient input into resident selection
4. Co-design of visible clinic change

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BACKGROUND

- Patient-centered care is key to healthcare quality
- 1/3 of patient-centered medical homes had a Patient Advisory Council (PAC)¹
- Many high-functioning PACs at Community Health Centers (CHCs)²

Patient Care,
Staff Culture

Clinic Workflow,
Space



- 2024 ACGME Requirement for Family Medicine Residencies³

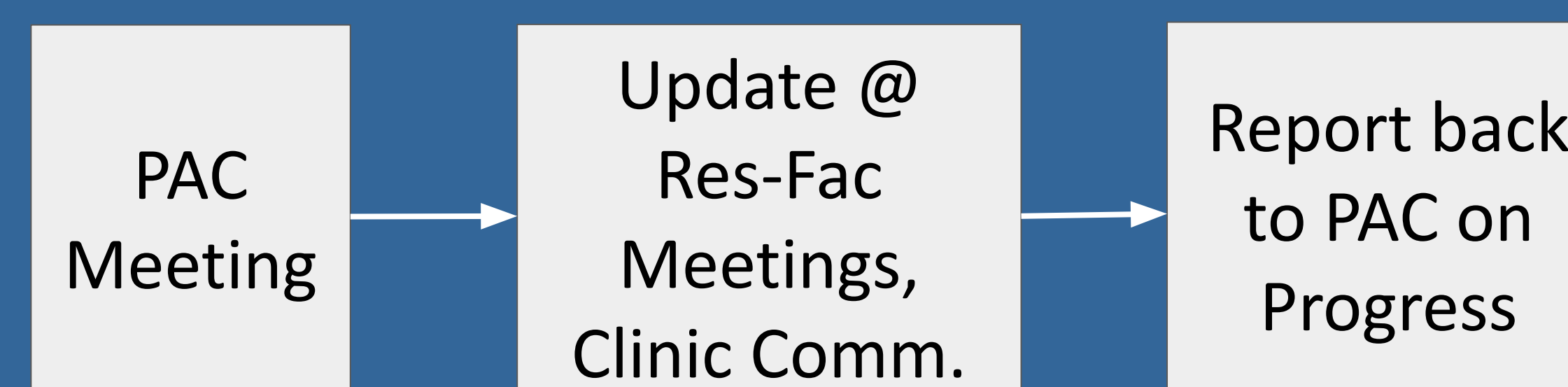
PATIENT ADVISOR ROLE

- Recruit: Paper Survey, Flyers, Referral
- Brief Screening/Orientation
- Pool of ~30-50 patients to draw from
- Representativeness
 - Equal number English, Spanish
 - Pregnant patients, Pediatric Patients, Disabled Patients, Working Families
- 18 month terms
 - Rolling Recruitment



PATIENT ADVISORY COUNCIL DESIGN

- Quarterly Meeting (2 hour)
 - 15 Patient Advisors
 - 1 Clinic Staff rep
 - 1 Resident rep
 - 1 Faculty rep
- Simultaneous English-Spanish, consulting community experts (e.g. PTA meetings)
- Facilitator: Resident, then Patient
 - Translate Patient Experiences into Specific Project Ideas



MEASURING PROGRESS AND RESULTS

- PAC impact on patient's experience of being heard in their care?
 - Liekert Scale: "I feel that this clinic listens to my perspective as a patient"
 - Paper survey, multiple languages
- PAC Impact on patient-physician relationship?
 - Structured Interviews
 - Patient advisors, residents, faculty, and patient-facing staff
- Annual Symposium



FOLLOW-UP & CONTINGENCIES

- Residents required attendance during Community Medicine month, Annual Co-Chair
- Faculty Pillars
- Patient advisors: new recruitment cycle every 18 months, overlap with continuing advisors
- Expansion to other languages (e.g. Vietnamese)
- Future funds of Santa Clara Valley Health



COMMON PITFALLS

- Lack of Engaged Leadership²
- Lack of Diversity
- Fall-off of patients
- Barriers to Participation
 - Transportation
 - Childcare

REFERENCES

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3. Accreditation Council for Graduate Medical Education. "ACGME program requirements for graduate medical education in family medicine." *Family Medicine* (2024).